

Documentation Gap Analysis: TGA COVID-19 Vaccine Safety Monitoring Plan Audit

INDEPENDENT REGULATORY COMPLIANCE ANALYSIS

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Paul Rekaris
rekasp@protonmail.com

Executive Summary

Audit Scope & Methodology

Comprehensive evidence-based assessment of TGA's February 2021 COVID-19 Vaccine Safety Monitoring Plan implementation across 19 specific measurable outputs. Analysis based on TGA's own systematic searches conducted September 2024 under OAIC direction (FOI Act s55V(2)), Senate testimony, FOI responses, and public documentation spanning four years (2022-2025).

3

Fully Implemented (16%)

10

Partial/Undocumented (53%)

6

Not Documented (31%)

Key Finding: While monitoring activities occurred (evidenced through 150+ weekly safety reports), systematic documentation of HOW enhanced COVID-19 vaccine safety monitoring was conducted is absent. Zero coordination protocols found linking AusVaxSafety's 6.8M surveys to TGA's 150 signal investigations. This creates an accountability gap: activity $\neq$ systematic implementation.

Complete Documentation Breakdown by Strategy:

- Strategy 1 (Reporting of AEFI): 40% fully documented
- Strategy 2 (Signal Detection and investigation): 0% fully documented as enhanced - most problematic
- Strategy 3 (Regulatory Actions): 0% fully documented
- Strategy 4 (Communication): 100% fully documented - best performance
- Strategy 5 (Collaboration): 0% fully documented
- Governance: 0% fully documented - critical gap

Audit Methodology Framework

Evidence-Based Approach

- Gap analysis comparing Feb 2021 Plan commitments against documented implementation
- Assessment against 17 specific measurable plan strategies*
- Evidence hierarchy: primary statutory (FOI, OAIC, TGA communications) > primary documentary (Senate testimony, QON 559, reports) > secondary documentary > tertiary contextual
- Objective evaluation: What can be demonstrated vs what cannot through temporal analysis over four years
- Falsifiable and replicable research framework

Primary Evidence Sources

- TGA OAIC Submission MR22/00538 (Sept 2024): Systematic searches organised by implementation plan objectives under legal obligation FOI Act s55V(2)
- Senate testimony (9 Oct 2025): Official statements under parliamentary privilege; QON 559 (Oct 2025)
- FOI responses (2022-2025): Formal government records spanning 4 years
- TGA public documentation: 150+ weekly safety reports, regulatory decisions, annual reports

Audit methodology applied systematic, evidence-based principles consistent with ISO 19011 (management systems auditing), ISO 15489 (records management), and Australian National Audit Office compliance audit standards

Implementation Status Classification Criteria

- ✓ Fully Implemented: Multiple forms of evidence, publicly accessible, systematic documentation exists, can be independently verified
- ⚠ Partially Implemented/Undocumented: Activities occurred (demonstrated through outputs) but implementation documentation absent, cannot verify systematic vs ad hoc approach
- ✗ Not Documented: No evidence of systematic implementation or documentation, no documents found in TGA searches despite claimed activities

*Two governance outputs were assessed alongside the Plan's 17 pharmacovigilance strategies, making 19 outputs audited in total

Evidence Hierarchy

TIER 1: Primary Statutory Evidence

FOI decisions (including FOI 3643), OAIC submissions (MR22/00538, MR25/01153), and direct TGA communications to the applicant. Formal statutory positions under the FOI Act 1982 and OAIC oversight.

TIER 2: Primary Documentary Evidence

Senate Community Affairs Legislation Committee testimony under parliamentary privilege (9-10 October 2025); Senate Question on Notice 559 (SQ25-001584); TGA Annual Reports 2022-23 and 2023-24; published COVID-19 vaccine safety reports; the COVID-19 Vaccine Safety Monitoring Plan (February 2021).

TIER 3: Secondary Documentary Evidence

Partner organisation reports (NCIRS AEMS COVID-19 vaccines annual report; AusVaxSafety surveillance data); Australian Immunisation Register data; publicly accessible surveillance data and performance metrics.

TIER 4: Tertiary Contextual Evidence

International regulatory guidance (EMA, MHRA, FDA enhanced monitoring frameworks); pharmacovigilance standards (ICH E2E signal detection; CIOMS multi-source integration); comparative regulatory frameworks.

*Higher-tier evidence takes precedence in case of conflict. All findings are anchored in Tier 1 or Tier 2 evidence.
Findings rest on the TGA's own searches and statements, not the auditor's interpretation.*

Key Assumptions & Limitations

Key Assumptions

1. Good Faith Searches: TGA's September 2024 searches under OAIC direction conducted in good faith using appropriate search terms
2. System Coverage: TRIM and Promap document management systems contain implementation documentation if it exists
3. Classification Framework: TGA's own plan objective classification (used Sept 2024) is appropriate framework
4. Absence Significance: Zero search results indicates absence of systematic documentation, not mere difficulty locating*
5. Search Comprehensiveness: Given systematic searches under legal obligation over four years reviewing 2,218+ pages, probability that comprehensive documentation exists but remains unlocated is negligible
6. Activity vs Documentation: Absence of documentation doesn't mean activities didn't occur, but does mean implementation of enhanced systemic post-market safety monitoring cannot be verified
7. Temporal Validity: Evidence from 2021-2025 remains valid indicator of implementation patterns

Limitations

1. Search Completeness: Reliant on TGA's own searches; cannot independently verify all TGA systems were searched
2. Document Sampling: Analysis based on documents TGA identified; search categories, additional documents may exist but not located
3. Informal Documentation: Assessment focuses on systematic documentation; informal records or institutional knowledge not captured
4. Verification Constraints: Cannot independently audit TGA's internal document management practices
5. Temporal Gaps: Analysis covers 2021-2025; implementation activities may have been documented after September 2024 searches
6. Legal Interpretation: Assessment identifies patterns and contradictions; does not constitute legal determination of statutory breach (requires judicial assessment)

*No documentation identified in TGA systematic searches, FOI responses, or public records

TGA's Regulatory Guidance - Provisional Approval

TGA Provisional Registration Transition Guidance

TGA's own published guidance establishes requirements for provisional approval pathway

TGA Provisional Approval Requirements

Enhanced Post-Market Safety Monitoring Obligation

Provisional approval is granted where further data is needed. Enhanced post-market monitoring applies as a condition of provisional registration, with sponsors expected to demonstrate safety surveillance beyond routine monitoring.

Risk Management Plans

Sponsors must submit detailed risk management plans setting out specific monitoring activities, timelines, and deliverables, addressing the uncertainties in preliminary clinical data.

Additional Studies Required

The TGA may require sponsors to conduct additional studies during the provisional period to address data gaps. Confirmatory data must establish the relationship between preliminary outcomes and clinical benefit.

Performance Conditions

Provisional registration is time-limited on the ARTG and subject to conditions that must be satisfied. Transition to full registration requires the sponsor to demonstrate that those conditions have been met.

Regular Reporting

Sponsors must provide regular safety and efficacy updates throughout the provisional period, with closer coordination across clinical, nonclinical, quality, and RMP evaluation areas.

TGA Evaluation and Delegate Satisfaction

The TGA evaluates the data submitted by the sponsor, and the Delegate must be satisfied that provisional conditions are met before granting transition to full registration. The TGA may seek external expert advice more frequently during the provisional period.

TGA's Accountability Gap

No Enhanced COVID-19 Vaccine Safety Monitoring Evidence

TGA published its COVID-19 Vaccine Safety Monitoring Plan (Feb 2021) committing to 'enhanced' monitoring. TGA investigated 148–150 signals and took 57 actions (Senate/QON 559, Oct 2025), but officials characterised this as 'day-to-day processes'. No documentation shows activities were enhanced beyond standard pharmacovigilance, and no audit trail links specific signals to specific actions, as expected under international good practice (ICH E2E, CIOMS).

No Plan-Level Transition Verification

Comirnaty (July 2023) and Spikevax (April 2023) transitioned on the sponsor's Category 1 application and benefit-risk data. The burden sits with the sponsor to demonstrate compliance. No documentation evaluating either transition against the Feb 2021 Plan's enhanced-monitoring conditions was located.

Contradictory Characterisation

Plan (Feb 2021): 'enhanced' monitoring. Senate testimony (Oct 2025): 'day-to-day processes'. These cannot both be operationally true as stated. If day-to-day satisfied 'enhanced', the distinction underpinning provisional approval has no content.

Sponsor-Demonstrated Compliance, No Plan Trail

TGA guidance gives provisional medicines 'high priority' under an enhanced framework, with the burden on the sponsor to 'continue to demonstrate' compliance or registration lapses. No documentation was located showing how the Plan's enhanced conditions were assessed at framework level during the provisional period or at transition.

Unverifiable Against the Plan's Commitments

The Feb 2021 Plan was the basis for enhanced monitoring across 68.4 million provisional doses. On the documentary record — FOI, OAIC-directed searches, published reports — implementation of that enhanced framework cannot be independently verified.

Provisional Approval Lifecycle - Verification Gap

Provisional Approval Lifecycle Framework Legislative Requirements (TGA Act 1989)

S 22C-s 22D: Determination Phase

Sponsor applies for a provisional determination (s 22C); the TGA assesses eligibility and may grant a provisional determination (s 22D), based on preliminary data and development stage.

S 23AA & s 28(2A)(aa): Provisional Registration

An application with an in-force provisional determination is treated as an application for provisional registration (s 23AA). The TGA may impose conditions requiring ongoing evidence generation, pharmacovigilance, and post-market monitoring during the provisional period.

s 29: Extension of Provisional Registration

Provisional registration is time-limited. Up to two extensions of two years each are permitted (s 29(4)), to a six-year maximum (s 29(8A)).

s 23 & s 25: Transition to Full Registration

Transition requires the sponsor to lodge a new application (s 23). The TGA evaluates the clinical dataset (s 25) to determine whether the medicine meets the baseline standards of safety, quality and efficacy required for full ARTG registration.

TGA Guidance Requirements

Risk Management Plans (RMPs)

Sponsors must submit detailed RMP outlining safety specification, pharmacovigilance activities, risk-minimisation measures, timelines and milestones for provisional period.

Transition to Full Registration

Requires the sponsor to demonstrate that provisional conditions have been met, including completion of confirmatory studies imposed as conditions of registration and delivery of updated safety and efficacy data. The TGA evaluates the transition application against the standards for full registration.

Scale and Significance

68.4 million doses (91.2% of rollout) administered under provisional approval between February 2021 and July 2023—Australia's largest deployment of provisionally approved medicines.

Evidence Gap: No Verification Documentation Found

MR22/00538 Searches (September 2024)

531+ TRIM containers searched using eight Plan-aligned terms under OAIC direction. No documents were located demonstrating:

- Verification that enhanced monitoring conditions satisfied before transitions
- TGA's documented assessment of whether provisional conditions met
- Decision-making criteria or rationale for granting full approval
- Verification checklist or evaluation of implementation against February 2021 Safety Monitoring Plan

FOI 25-0166 (June 2025)

399 Plan-aligned documents identified across 12 categories. The TGA refused the request under s 24AA(1)(b) (inadequate identification), but relied on ~177 hrs of s 24AA(1)(a) decision-making time against ~3 hrs of identification — conflating two statutory tests.

AusPAR Analysis: No Verification Evidence

Comirnaty AusPAR (July 2023): Documents conversion to full registration (TGA decision 13 July 2023, ARTG entry 14 July 2023) but contains no evaluation of Safety Monitoring Plan implementation. No verification checklist showing provisional conditions met. No plan-level governance documentation.

Spikevax transition (April 2023): TGA media release contains no reference to Plan verification.

Pattern: AusPARs summarise clinical data, RMP and conditions but do not identify any verification checklist or evaluation of enhanced monitoring implementation.

Critical Accountability Gap: No evaluation against the February 2021 Plan's enhanced-monitoring conditions is documented in the public or FOI record. While the TGA evaluated the sponsors' transition applications against the general full-registration standards (safety, quality, efficacy under s 25), no documented process shows how the Plan's enhanced-monitoring commitments were assessed before transition. The evidentiary basis for verifying those specific conditions across 68.4 million provisionally approved doses cannot be independently established from the available record.

International Comparison - Regulatory Transparency Standards

TGA's disclosable record omits documentation routinely published by peer regulators

FDA Practice (US)

- Approval letters detailing basis for decision
- Comprehensive review memoranda from FDA medical officers
- Clinical study data summaries
- Post-authorisation safety monitoring reports
- Briefing documents for advisory committee meetings
- Live streaming of Public advisory committee proceedings (VRBPAC)
- Example: Comirnaty approval (Aug 2021) = 30-page approval letter + hundreds of pages of supporting documentation

EMA Practice (EU)

- European Public Assessment Reports (EPARs) for each transition
- Pharmacovigilance Risk Assessment Committee (PRAC) recommendations
- Periodic safety update reports
- Committee for Medicinal Products for Human Use (CHMP) assessment reports
- Comprehensive safety monitoring results
- Independent expert committee review documentation

TGA Practice (Australia)

Published Documentation:

- Australian Public Assessment Reports (AusPARs) – published summaries of TGA's evaluation of sponsor submissions, including clinical data, key benefit–risk considerations
- Media releases announcing transitions and related decisions

Notably Absent (in public and FOI record):

- X TGA's own post-market monitoring results and analysis
- X Systematic evaluation of compliance with provisional-approval conditions, including in TGA annual performance reports
- X Safety Monitoring Plan implementation assessment
- X Documentation of how the Plan's enhanced monitoring conditions were verified

No independent expert review:

The Comirnaty transition was not referred to the Advisory Committee on Vaccines (AusPAR, Advisory Committee considerations)

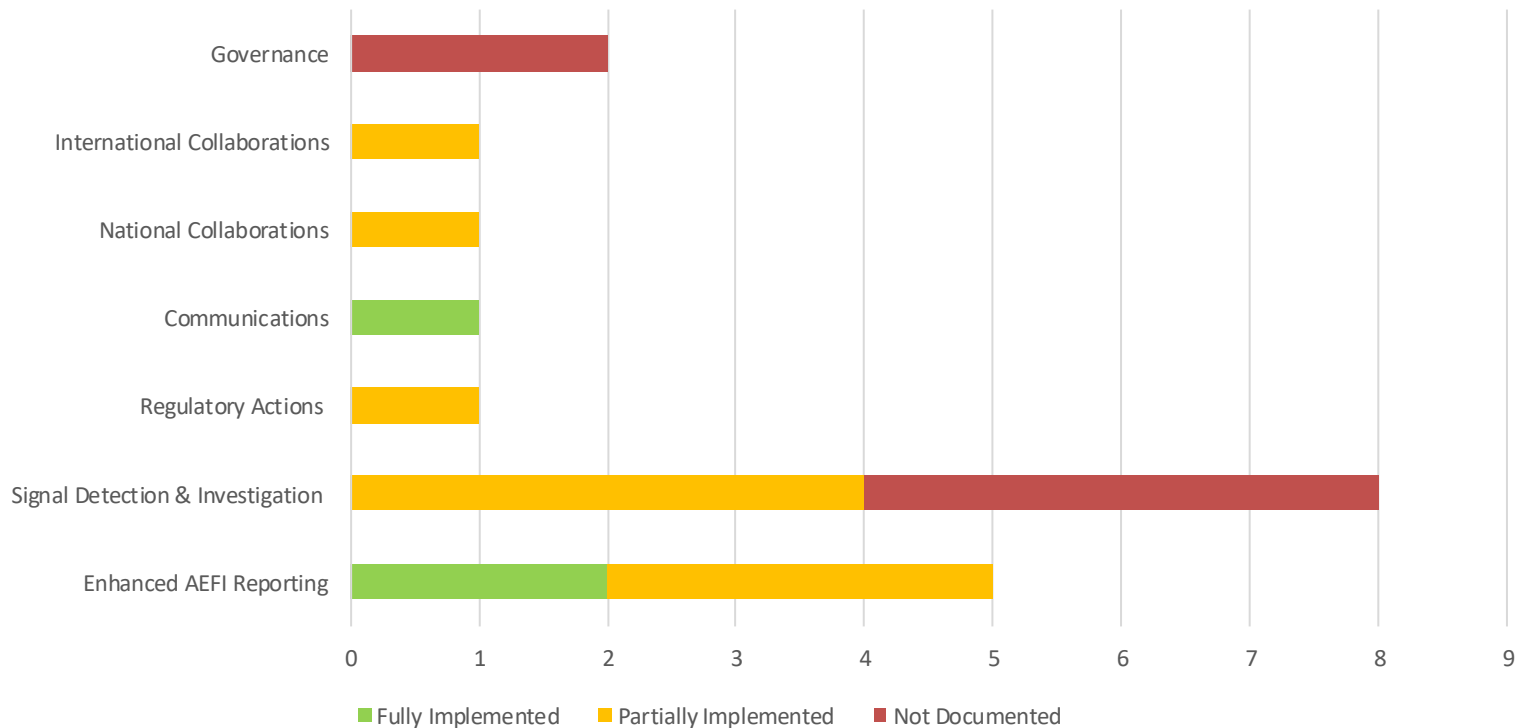
Implementation Status by Safety Plan Outputs

Strategy	Total outputs	Implemented	Partial	Not Documented
1: Enhanced reporting of Adverse Events Following Immunisation	5	2 (40%)	3 (60%)	0 (0%)
2: Enhanced safety signal detection and investigation	8	0 (0%)	4 (50%)	4 (50%)
3: Actions in response to safety concerns	1	0 (0%)	1 (100%)	0 (0%)
4: Communications	1	1 (100%)	0 (0%)	0 (0%)
5: Collaborations	2	0 (0%)	2 (100%)	0 (0%)
Governance and performance measurement*	2	0 (0%)	0 (0%)	2 (100%)
Total	19	3 (16%)	10 (53%)	6 (31%)

*Assessed using ISO 19011:2018 audit method. Two governance outputs (GOV. 1, GOV.2) added against PGPA Act 2013 ss 37-39, additional to the Plan's 17 strategies.

Fully documented: fully evidenced in official documentation, Partial: some activity, but in incomplete documentation; Not Documented: no evidence found

Implementation Status by Safety Plan Strategy



AusVaxSafety: Active Surveillance Without Documented Integration

COVID-19 vaccine safety data – at a glance

As at 14 April 2025

6,887,520

COVID-19 vaccine safety surveys completed*

106,929

COVID-19 vaccine safety surveys completed by Aboriginal and Torres Strait Islander people*

43.5%

reported at least one adverse event†

0.9%

reported visiting a GP or emergency department

* Surveys sent on day 3 post vaccination.

† Adverse events are self-reported, have not been clinically verified, and do not necessarily have a causal relationship with the vaccine.



 **AusVaxSafety**
An NCIRS-led collaboration

Critical Documentation Gap:

- Zero coordination protocols found linking AusVaxSafety data to TGA's 150 signal investigations
- No documented integration between active surveillance (6.8M surveys) and passive surveillance (DAEN database)
- No audit trails showing how population-level adverse event data informed TGA's 57 regulatory actions
- Cannot verify disposition of 93 signals resulting in "no further action"

Why This Matters:

- Active surveillance promised as core pillar of TGA-led pharmacovigilance in Australian COVID-19 Vaccination Policy
- Strategy 2.4 commitment: "Collaborate with AusVaxSafety to receive active surveillance information and coordinate safety signal detection and investigation activities"
- Without documented integration frameworks: Cannot verify whether 62,000 medical attention reports and 3 million adverse event reports systematically informed signal prioritisation and regulatory decisions
- Enhanced monitoring standard: International pharmacovigilance standards (ICH E2E, CIOMS) require documented coordination protocols with audit trails—not parallel operation of surveillance

Key finding: Operational capability ≠ enhanced COVID-19 vaccine safety monitoring framework – infrastructure functioned, but systematic integration is undocumented and remains unverified.

Accountability Implications of Documentation Gaps

Absence of verifiable documentation creates serious accountability deficits across multiple dimensions

Legal Basis Questioned

The provisional pathway permits registration based on less mature clinical evidence, with ongoing evidence generation and enhanced post-market monitoring during the provisional period. If the Plan's enhanced monitoring cannot be demonstrated from the record, questions arise regarding: the evidentiary basis on which the enhanced-monitoring commitments underpinning the 2021 provisional approvals were met, the 2023 transition decisions, and accountability of the provisional approval pathway.

Ministerial Accountability Deficit

The Minister for Health is accountable to Parliament for the TGA's regulatory framework. Where the documentary record does not show what evidence supported transition decisions, how the Plan's enhanced-monitoring conditions were assessed, or whether the promised enhanced monitoring was implemented, the basis on which Parliament can be assured that these commitments were met cannot be independently verified from available records. TGA officials gave Senate evidence (Oct 2025) of 'some difficulty' producing documents by Plan objective, consistent with this documentary gap.

Public Trust Implications

Australians received COVID-19 vaccines under a provisional framework that explicitly committed to enhanced safety monitoring, and TGA communications emphasised rigorous oversight. Where the record does not document that the Plan's enhanced monitoring was implemented, the public assurances of heightened oversight cannot be independently verified, and the commitments made when reduced pre-market evidence was accepted cannot be shown, on the record, to have been met.

Provisional Approval Framework Integrity

Framework integrity depends on verifiable evidence that enhanced post-market monitoring occurred, that compliance with the Plan's enhanced-monitoring conditions was assessed, and that this assessment was documented before transition. For the COVID-19 vaccine transitions, none of these can be independently verified from the public or FOI record. The audit does not find that oversight did not occur, but that the enhanced monitoring on which the pathway's reduced pre-market requirements rest cannot be verified from available records.

Accountability Implications of Documentation Gaps

Absence of verifiable documentation creates serious accountability deficits across multiple dimensions

Parliamentary Oversight Without Documentation

In Senate estimates the TGA could not produce implementation documentation for scrutiny: officials characterised monitoring as 'day-to-day processes' and acknowledged 'some difficulty' producing documents by Plan objective, which sits against the 'enhanced' monitoring commitment; FOI requests were welcomed but no implementation documentation was produced; and no completed implementation review was identified, despite a commitment to review recorded in a February 2022 email. Parliament questioned the TGA, but the documentary basis for assessing whether commitments were met was not available.

International Benchmarks

Australia's regulatory practice is expected to align with international pharmacovigilance good practice (ICH E2E, EMA GVP) and with the transparency standards peer regulators publish. Where documentation meeting these benchmarks cannot be produced: the alignment between Australia's disclosable record and these benchmarks cannot be demonstrated; the TGA's own COVID-19 monitoring is not documented to the standard it requires of sponsors; and the transparency the pathway relied upon cannot be independently verified.

TGA Obligations Under Commonwealth Law – Conformity Assessment

Statute / Provision	Requirement	Documentary Finding	Conformity*
PGPA Act 2013, s 37	Records must properly record and explain the entity's performance in achieving its objectives	No documentation tracking performance against Plan objectives located (GOV.1).	Non-conforming
PGPA Act 2013, s 38 / Rule s 16F(2)	Duty to measure and assess performance; results reported in performance statements	No performance measurement against Plan outputs located (GOV.2)	Non-conforming
Archives Act 1983, s 24	Commonwealth records must not be disposed of except as authorised	If records existed and were not retained: potential concern. If never created: records gap. The record does not distinguish.	Indeterminate
Therapeutic Goods Act 1989, ss 22D, 23AA, 28(2A)(aa)	Provisional determination and registration; conditions requiring ongoing evidence and post-market monitoring may be imposed	The Plan's enhanced-monitoring framework cannot be demonstrated to have been operationalised; no evaluation against the Plan's conditions located	Non-conforming
Therapeutic Goods Act 1989, ss 23, 25	Transition requires a full-registration application, evaluated against efficacy, safety and quality standards	Transition evaluated under s 25 against general standards; no evaluation against the Plan's enhanced-monitoring conditions located	Conforming †
FOI Act 1982, s 24A	Take all reasonable steps to locate documents; refuse only where they cannot be found or do not exist	AICmr 54: TGA took all reasonable steps; documents cannot be found or do not exist	Conforming
FOI Act 1982, s 24AA(1)(a) vs (1)(b)	Practical refusal must cite the correct ground	June 2025: refusal cited s 24AA(1)(b) (inadequate identification), but relied on ~177 hrs of decision-making time relating to s 24AA(1)(a) (diversion of resources), against ~3 hrs of identification. Two separate statutory tests conflated.	Under review

† Conforming to the general full-registration standard (s 25); that standard does not include assessment against the February 2021 Plan.

*Assessment of apparent non-compliance patterns, subject to formal investigation and legal determination.

International Pharmacovigilance Standards - Context

IMPORTANT: These are NOT legally binding requirements but provide context for assessing TGA's practices against international norms and TGA's stated commitments to align with these standards

Standard	Status/Basis	Key Requirement	TGA Gap
ICH E2E Pharmacovigilance Planning	Recognised good practice; TGA (adopted June 2005)	Requires structured action plans with: safety issue, objective, actions, rationale, monitoring, milestones	No documentation identified of structured action plans per ICH E2E requirements. No milestone tracking documented.
CIOMS WG VIII	Internationally recognised good practice	Traceable audit trails from signal detection to decision. Documented signal investigation protocols and assessment procedures.	No audit trails linking 150 investigated signals to 57 regulatory actions. No documented signal investigation protocols identified.
EMA GVP Module I	Reference standard adopted in TGA guidelines (August 2025)	Mandatory requirements for quality system documentation, training records, performance monitoring	No documentation identified meeting EMA GVP Module I quality system requirements for COVID-19 monitoring.
PIC/S Good Vigilance Practices	TGA is PIC/S member (applied here as a benchmark)	Comprehensive pharmacovigilance system documentation, quality management, performance indicators	Cannot demonstrate PIC/S GVP compliance - no systematic documentation of quality management.
WHO Pharmacovigilance Framework	Recognised international framework	National pharmacovigilance plans should include objectives, activities, timelines, performance measurement	No documented performance measurement against plan objectives. No timeline documentation.

Systemic Issues Identified - Pattern Analysis

1

Documentation vs Activity Gap

Activities occurred (150+ weekly reports, 150 safety signals investigated, 57 regulatory actions taken) but systematic documentation of HOW activities were conducted is absent. Creates verification impossibility: outputs visible, processes invisible.

2

Enhanced vs Standard Monitoring

Senate testimony: 'day-to-day processes'. Plan commitment: 'enhanced monitoring'. Cannot reconcile these characterisations. If day-to-day = enhanced, provisional approval framework meaningless.

3

Governance Vacuum

Zero governance documentation: no implementation oversight, no performance review, no accountability framework. Feb 2022 review promise never materialised. Four-year accountability gap.*. Senate testimony (Oct 2025): A/g Medical Officer admitted 'some difficulty' producing documents demonstrating Safety Plan implementation - confirming records were never systematically organised by objective.

4

Records Management Failure

Systematic absence of implementation documentation potentially indicates either: (a) never created = PGPA/Archives failure, or (b) created then destroyed = Archives Act breach. Either way, systemic records management failure.

5

Temporal Contradictions in FOI

TGA demonstrated capability (Sept 2024: 3 hours to organise 2,218+ pages by objectives) then claimed impossibility (June 2025: 'too subjective'). Pattern is inconsistent with the earlier demonstrated capability

6

Provisional Approval Integrity

Cannot verify enhanced-monitoring conditions were assessed against the Plan before full approval was granted. Framework requires verification; verification requires documentation; documentation absent. Framework integrity potentially compromised.

*Cannot demonstrate on public record

Key Findings - Critical Oversight Failures Uncovered

TGA published 150+ weekly safety reports but cannot produce documented evidence distinguishing enhanced monitoring from routine surveillance. Officials characterised processes as 'day-to-day' without systematic tracking. The absence of documented coordination protocols, investigation frameworks, and governance mechanisms means the enhanced monitoring underpinning provisional approval cannot be verified from the record, potentially undermining provisional approval framework integrity and public trust.

1. No Evidence of Systematic Enhanced COVID-19 vaccine safety Monitoring

TGA promised enhanced surveillance beyond routine processes as provisional approval condition. No documented evidence found despite reviewing 2,218+ pages (Sept 2024) and identifying 399 Plan-aligned documents (FOI 25-0166) refused for processing. Officials admitted 'day-to-day processes' without systematic tracking. Zero coordination protocols linking AusVaxSafety's 6.8M surveys to TGA's 150 signal investigations. Enhanced vaccine safety monitoring unverified. (Strategy 2; Senate Oct 2025; QON 559 FOI 25-0166)

2. Complete Absence of Governance and Accountability Frameworks

No documented oversight, performance measurement, or governance mechanisms exist. Searches found zero governance documents. Over 3 years since Feb 2022 review promise with no formal reviews, evaluations, or audit trails. TGA 2023-24 Annual Report contains zero Plan references despite 68.4 million doses, creating profound accountability gap. (Outputs GOV.1, GOV.2; TGA Annual Report 2023-24)

3. No Documented Risk-Based Signal Detection Systems

TGA investigated 150 safety signals and took 57 regulatory actions (Senate Oct 2025), yet cannot produce frameworks, protocols, or methodological standards for risk prioritisation, signal investigation, or causality assessment. No documentation links signals to actions or provides audit trails required by international standards (ICH E2E, CIOMS). Zero coordination protocols between AusVaxSafety's surveys and signal investigations. Searches found 0 documents for outputs 2.2, 2.3, 2.4. (Outputs 2.2, 2.3, 2.4)

4. Transparency Failures Undermine Public and Parliamentary Oversight

Despite identifying 399 Plan-aligned documents (FOI 25-0166) and reviewing 2,218+ pages (Sept 2024), TGA consistently refused release citing 'subjectivity' and resource constraints. TGA used identical document classifications systematically for OAIC (Sept 2024) then claimed same classifications 'too subjective' for citizen requests (June 2025). Contradictory statements erode trust and impede parliamentary scrutiny. FOI delays exceeded 1,400 days.

In plain terms: provisional approval traded reduced pre-market evidence for a promise of enhanced post-market safety monitoring. Australians have no way to verify from the record that this enhanced monitoring was delivered – or that it differed from routine surveillance at all.

Required Reforms - Restoring Framework Integrity

Public communication (Strategy 4) achieved full implementation; the documentation and governance domains did not. The audit finds systemic gaps requiring substantial reform to make the provisional approval pathway accountable and verifiable in future emergencies.

These same documentation and governance failures will recur in the next pandemic unless addressed. Provisional approval framework integrity depends on learning from COVID-19 experience and implementing reforms for future emergency approvals to ensure public trust.

1. Governance & Accountability

Oversight mechanisms should be strengthened to ensure emergency response frameworks have clear performance measurement and accountability pathways for vaccine safety monitoring. Independent audit and verification processes should underpin regulatory transitions and approvals to demonstrate that enhanced monitoring standards have been satisfied.

2. Documentation & Records Management

Documentation standards should demonstrate how enhanced monitoring differs from standard practice, with protocols, criteria, and key decisions systematically recorded and published. Compliance with mandatory creation, retention, and archiving obligations should be maintained, including during emergencies.

3. Systems for Risk Assessment & Verification

Robust frameworks should be developed for risk-based signal detection, prioritisation, investigation, and causality assessment, with regular scientific reviews and transparent methodologies to support decision-making.

4. Transparency & Public Engagement

Provisional approval compliance should be supported by public access to monitoring data, processes, and methodologies, alongside timely responses to FOI and parliamentary requests. Leadership should be accountable for upholding transparency, documentation and compliance obligations throughout regulatory processes.

5. Emergency Preparedness

Emergency preparedness should include scalable systems for rapid safety monitoring, strong data handling, and requirements for ongoing system improvement for future health emergencies when public trust is most critical.